DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
4212 North 16th Street
Phoenix, Arizona 85016

**MEMORANDUM**

**To:**
Endreya McCabe, Director of Quality and Performance Improvement
Clinton Pope, Chief Quality Officer
Human Resources Office

**From:**
Kevin D. Turnage, MSN Ed., RN, NE-BC, NHDP-BC, NPD-BC
Policy and Procedure Nurse Consultant

**Date:** May 21, 2025

**Subject:** Formal Objection and Legal Clarification of Authority to Reject Noncompliant Policies

## I. Purpose

This memorandum documents and responds to a statement made by Mr. Clinton Pope and Ms. Endreya McCabe during a Microsoft Teams meeting held from **10:30 a.m. to 11:30 a.m. on May 21, 2025**, in which I was explicitly told:

**"You do not have the authority to reject a policy."**

This statement is incompatible with the responsibilities defined in my official Position Description (PX3147), as well as with applicable federal law, whistleblower protections, and Indian Health Service (IHS) policy governance standards. The purpose of this memorandum is to formally dispute that assertion, request clarification in writing, and reaffirm my legal and protected responsibility to ensure compliance in the development and implementation of organizational policies.

## II. Legal Authority and Federal Obligations

As a federal employee in a policy and compliance role, I am legally obligated to act when proposed policy fails to meet minimum regulatory standards. This includes:

- **Federal regulations** (e.g., 42 C.F.R. § 482.21 – CMS Conditions of Participation);
- **Accreditation standards** (e.g., The Joint Commission);
- **Evidence-based clinical practices** essential to patient safety and risk mitigation.

My duty to raise such concerns and prevent the adoption of noncompliant policies is protected by:

- **5 U.S.C. § 2302(b)(8)** – Prohibits retaliation for disclosing violations of law, gross mismanagement, or specific risks to public health or safety;
- **41 U.S.C. § 4712** – Protects disclosures concerning performance failures in federally funded healthcare programs;
- **IHS Circular 18-04** – Requires prompt identification and correction of patient safety and policy deficiencies.

## III. Scope of Duties – Position Description PX3147

My Position Description (PX3147, GS-0610-12) specifically defines my authority and obligation to evaluate, revise, and ensure legal/regulatory integrity in organizational policy. Notable language includes:

- *"The incumbent advises hospital departments and services on regulatory and policy requirements and participates in the development, evaluation, and revision of hospital-wide policies and procedures."*
- *"Independently reviews Joint Commission, CMS, OSHA, and other regulatory standards... and provides guidance on how to incorporate these standards into PIMC's policy infrastructure."*
- *"Serves as a key resource in identifying deficiencies and gaps in existing policies and recommends corrective action consistent with evidence-based practice and federal law."*
- *"Analyzes organizational policies to assure that the content is valid, consistent, and without conflicting content, assuring that the procedures describe actual or intended practice."*

This scope of duty makes clear that I am not only permitted—but required—to identify and prevent the approval of any policy content that would expose the organization to compliance or clinical risk.

## IV. Additional Classification Factors Supporting Independent Authority

PX3147's classification under the Office of Personnel Management's Factor Evaluation System (FES) further confirms that I am authorized to exercise expert judgment in all phases of policy development and review:

- **Factor 1 – Knowledge (Level 1-7):** Requires broad and advanced knowledge of clinical standards, federal regulatory frameworks, and accreditation policies sufficient to develop, revise, and ensure lawful implementation of healthcare policy.
- **Factor 2 – Supervisory Controls (Level 2-4):**
    - *"As an expert, the incumbent works independently to plan and complete work assignments… interprets regulations on his/her own initiative… applies new methods to resolve complex and/or intricate, controversial, or unprecedented issues… and resolves most of the conflicts that arise. The supervisor does not usually review methods used."*
    - This explicitly affirms that I am not required to seek permission to interpret regulations, raise objections, or prevent the advancement of noncompliant policy content.

- **Factor 3 – Guidelines (Level 3-4):** Involves adapting incomplete or conflicting regulatory guidelines and exercising independent judgment in how to apply or reconcile them within organizational policy.
- **Factor 4 – Complexity (Level 4-4):** Involves development of entirely new or adapted processes to address novel, controversial, or unprecedented problems related to policy integrity.
- **Factor 5 – Scope and Effect (Level 5-4):** The work affects hospital-wide clinical operations, regulatory readiness, and directly supports safe patient care and organizational accreditation.
- **Factors 6 & 7 – Personal Contacts and Purpose:** Requires interaction with accrediting agencies, CMS, IHS Headquarters, and internal leaders to develop policy language, resolve disputes, and ensure compliance alignment.

## V. Legal and Organizational Risk

Any directive that denies or attempts to limit my authority to reject policy content on compliance grounds creates organizational and legal exposure, including:

- Violations of **federal whistleblower protection laws**;
- Deficiencies during **CMS or Joint Commission surveys**;
- Legal liability for knowingly implementing flawed or noncompliant policy language;
- Institutional failure of internal controls.

## VI. Request for Written Clarification and Affirmation

Given the seriousness of the statements made on May 21 and their implications, I respectfully request the following **in writing**:

1. **Confirmation** that I am authorized and expected to raise objections to, and recommend rejection of, noncompliant or contradictory policy content in accordance with PX3147 and applicable law;
2. **Affirmation** that no retaliatory action or limitation of duties will result from my good-faith performance of those responsibilities;
3. **Assurance** that policy development at PIMC is a compliance-governed process and not discretionary.

**If any agency official believes that I do not possess this authority, I respectfully request a formal legal determination be issued in writing—signed by either the Area Legal Counsel or the Office of General Counsel—citing the specific statute, regulation, or policy that removes or limits this authority as described in my Position Description and applicable federal law.**

**Supplemental Note:**
In 2024, I formally requested a **desk audit** of my position due to concerns that the duties I was performing were not accurately reflected in my classification or title. To date, I have not received any written acknowledgment, status update, or outcome of that request. I ask that this memo be considered in conjunction with that prior request and that the audit be conducted without further delay.

## VII. Protected Disclosure Statement

This memorandum constitutes a **protected disclosure** under **5 U.S.C. § 2302(b)(8)** and **41 U.S.C. § 4712**, submitted in good faith to document interference with a federally protected compliance function and associated organizational risk. Any retaliatory conduct or adverse action in response would constitute a violation of federal law.

**Respectfully submitted,**

**Kevin D. Turnage, MSN Ed., RN, NE-BC, NHDP-BC, NPD-BC**
Policy and Procedure Nurse Consultant
Phoenix Indian Medical Center

**CC:** U.S. Office of Special Counsel (OSC)